

FairBanks Community Outreach Program

(FairBanks Outreach)

Sports & Family Health: A New Component

Concept Note & Partnership Proposal

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1. Executive Summary

FairBanks Medical Centre Limited is an established healthcare provider based in Kampala, offering 24/7 emergency care, diagnostics, pharmacy, maternal and child health, and specialist services to Uganda's urban and peri-urban communities. The Centre's community outreach initiative, the FairBanks Community Outreach Program (branded publicly as FairBanks Outreach), already runs six components: Mobile Medical Clinics, an Elderly Care Initiative, Health Education, Maternal & Child Health services, Free Vaccination Drives, and Vision & Dental Care, reaching rural and peri-urban communities, elderly citizens, mothers, children, and other underserved populations, in partnership with the Ministry of Health, Centenary Bank, Denk Pharma, Radio One, and the Uganda Heart Institute. FairBanks is also building FCHIP, the FairBanks Community Health Intelligence Platform, which turns outreach and clinic data into early warning and follow-up action for the communities it serves.

A conversation with a US based sports coach who runs his own sports initiative, and who is interested in funding community health work, has opened the opportunity to add a seventh component to FairBanks Outreach: Sports & Family Health. This concept note proposes a set of outreach camps, delivered at churches, sports centres, and schools across Kampala's peri-urban communities, that package first aid, ambulance standby, sports massage and physiotherapy, hydration and rehydration support, non-narcotic pain management, and triage and family-health screening into a single, welcoming outreach day with every visit captured on FCHIP for referral and follow-up, exactly as FairBanks Outreach's existing components already operate. Sport is the entry point for this new component; family health, delivered through FairBanks' existing outreach model, is the outcome.

Year 1 proposes a Kampala peri-urban pilot of 12 monthly camps — one per month, rotating across churches, sports centres, and schools, reaching an estimated 4,800 people. The fully costed Year 1 budget totals approximately USD 50,440, worked out to roughly USD 10.51 per person reached, and is presented in full line-item detail in the accompanying Excel workbook. FairBanks proposes a start date of September 2026, or October 2026 at the latest, in line with the funder's stated timeline. A strong Year 1 opens a credible, evidence-based path to expand first to other regions of Uganda, and then into Congo, Rwanda, Burundi, Tanzania, and potentially Malawi.

2. About FairBanks Medical Centre & the Outreach Program

FairBanks Medical Centre Limited was established to meet the growing healthcare demands of Uganda's urban and peri-urban communities. Its stated mission is to be “the most trusted provider of quality healthcare and pharmacy services,” grounded in innovation, compassion, and integrity, and delivering affordable, patient-centred care with professionalism and empathy. The Centre is registered with the Uganda Revenue Authority (TIN: 1053370026), with its principal place of business at Plot 1423 & 1425, Tirupati Road, Kololo, Kampala, and offers general outpatient and inpatient care, 24/7 emergency services with a trauma response team, advanced diagnostics, internal medicine and specialist consultations, paediatrics, obstetrics and gynaecology, ENT and ophthalmology, urology, physiotherapy and rehabilitation, a retail pharmacy, and vaccination programs. It works with CIC and AAR insurance, with Jubilee and Mutual coming online.

2.1 The FairBanks Community Outreach Program (FairBanks Outreach)

The FairBanks Community Outreach Program branded publicly as FairBanks Outreach is FairBanks Medical Centre's community outreach program, extending care “beyond our walls” to rural and peri-urban communities, elderly citizens, mothers, children, and other populations that cannot easily access medical facilities. It is already built around six components:

Component	What it delivers
Mobile Medical Clinics	Fully equipped units delivering free screenings, basic treatment, medication, health education, and vaccination drives to underserved communities
Elderly Care Initiative	Home visits by nurses, chronic disease management, medication assistance, and social support for seniors (delivered through the GeriCare Program)
Health Education	School programs, community workshops, nutrition education, and hygiene campaigns
Maternal & Child Health	Antenatal care, immunization campaigns, nutrition support, and new-mother education
Free Vaccination Drives	Child immunizations, HPV vaccination, and flu shots
Vision & Dental Care	Eye exams, glasses distribution, dental checkups, and oral health education

Recent FairBanks Outreach activity — delivered through these components — includes:

Camp / Activity	Date	Focus
Eye Camp	March 2026	Free vision screenings and assessments
Community Vaccination Drive	May 2026	Free vaccination against preventable diseases
Cardiology Camp	17–19 July 2026	Free heart screenings, ECGs, and consultations

These activities have been delivered in partnership with the Ministry of Health, Centenary Bank, Denk Pharma, Radio One, and the Uganda Heart Institute. The same kind of multi-sector partnership model this proposal extends to a sports-sector funder. This proposal does not create a new, separate program; it proposes Sports & Family Health as a seventh FairBanks Outreach component, run on the same outreach model, reporting into the same FCHIP platform, alongside the six above.

2.2 GeriCare: proof of a family-health lens

The Elderly Care Initiative is delivered through FairBanks' GeriCare Program, addressing Uganda's fast-growing elderly population (projected to reach 5.5 million by 2050) through home-based and facility-based services: routine nursing visits, chronic disease management, dementia and memory care, palliative care, caregiver training, and home-based geriatric assessments. GeriCare demonstrates that FairBanks Outreach already thinks in terms of whole family health across its components, not single disease campaigns, exactly the framing a Sports & Family Health component extends into a new setting.

2.3 FCHIP: the data backbone

FCHIP (the FairBanks Community Health Intelligence Platform) is described by FairBanks as “the technology layer that helps the whole loop learn and improve.” It already supports four stages of work: capturing structured health information in the field, AI-assisted risk scoring to flag rising disease patterns, generating alerts through worklists and dashboards, and enabling outreach response and follow-up. Its features include GIS mapping of case clusters by village or facility catchment, climate-data fusion for early warning (for example, fever clusters following heavy rain), offline-capable mobile forms for community health workers, and secure APIs that let existing clinic and hospital systems share data without being replaced. Community health workers (CHWs) and Village Health Teams (VHTs) receive field worklists; FairBanks teams see trend dashboards; programme managers monitor outreach indicators; and district partners can view shared evidence for planning.

3. Why Add a Sports & Family Health Component

FairBanks Outreach's six existing components already reach clinics, schools, homes, and mobile clinic stops. A conversation with a sports coach (Coach Dunbar) who runs his own initiative in the United States, someone who has built his career, and his platform, around sport has surfaced a gap none of the six quite cover: sports centres, playing fields, and the athletes, coaches, and youth leagues around them. Adding Sports & Family Health as a seventh component closes that gap, using the same outreach model FairBanks Outreach already runs elsewhere.

- Sport gathers whole communities, not just patients. A football pitch on a Saturday morning draws players, coaches, families, spectators, and vendors a natural, non-clinical setting where people who would never walk into a hospital will happily walk past a health tent.
- Coaches and athletes are trusted local voices. A message about hydration, injury care, or screening lands differently when it comes with the credibility of a sports figure rather than a clinic flyer.
- Combined with churches and schools, venues also used by this new component, sports centres complete family-unit coverage across all of FairBanks Outreach: schools reach children and, through them, parents; churches reach multi-generational households on weekends; sports centres reach youth, working-age adults, and athletes specifically.
- A visible sports-medicine offering massage, physiotherapy, hydration, injury care gives this component a natural, differentiated identity that speaks directly to the funder's own field, rather than a generic ask for “medical camp funding.” It gives the partner something concrete and personal to put his name behind.
- Partnership with a sports figure also opens doors beyond direct funding: athlete ambassadors, sports-centre venues at no cost, youth-league mobilization, and connections into diaspora and sports-philanthropy networks in the United States.

Sports & Family Health is designed to make that connection explicit, not a standalone program, but the seventh component of FairBanks Outreach, using sport as its entry point and family health as its outcome, on infrastructure (staffing model, FCHIP, partnership structure) FairBanks Outreach already has in place.

4. Program Design: Sports & Family Health Camps

Each Sports & Family Health camp is organized as a single-day, eight-station circuit that a family can move through together, the same outreach-day structure FairBanks Outreach's other components already use,

adapted to a sports setting. The stations translate the original service list: first aid, ambulance services, massage, drinks, pain relief, and triage into a coherent, professionally-run flow:

#	Station	What happens	Who it serves
1	Registration & FCHIP Intake	Digital registration on tablets; each participant gets a FCHIP record for follow-up	Everyone
2	Triage & Vitals	Blood pressure, temperature, weight/BMI, blood glucose screening	Everyone, especially adults 35+
3	First Aid & Trauma Care	Wound care, minor injury treatment, sports-injury first response	All ages, athletes especially
4	Ambulance Standby	Fully-crewed ambulance on site for emergency referral to FairBanks Medical Centre	Emergency cases
5	Sports Massage & Physiotherapy	Recovery massage, basic physiotherapy, injury assessment	Athletes, coaches, active adults, elderly
6	Pharmacy & Pain Management	Non-narcotic pain relief, essential medicines, dispensing (see Note, Section 11.2)	As clinically indicated
7	Hydration & Nutrition	Drinking water, oral rehydration drinks, nutrition education	Everyone, especially children & athletes
8	Family Health Education & Screening	HIV testing & counselling, family planning, maternal & child health, deworming referral	Families

This model keeps the emotional core the funder cares about: sport, movement, recovery, community while giving FairBanks' clinicians room to run a genuinely useful screening and referral service, all captured through FCHIP, so no visit is a dead end.

5. Target Communities & Venues

Camps rotate across three venue types, each chosen for a different reason:

- Churches: reliable weekend attendance across multiple generations of the same household, strong existing trust with community leaders.
- Sports centres and playing fields: direct alignment with the funder's interest, access to youth leagues, athletes, and coaches, and a natural home for the massage/physiotherapy station.
- Schools: access to children and, through school events and PTA days, their parents; a natural setting for deworming, vaccination catch-up, and nutrition education.

Site selection within Kampala's peri-urban areas will prioritise population density, distance from the nearest health facility, and existing FairBanks Outreach relationships (for example, the Kyebando-Kisalosalalo / Northern Bypass corridor, an established peri-urban outreach area), so that Year 1 sites can be confirmed quickly and camps can begin on schedule. We are already serving this area through our outreaches.

6. Geographic Scope & Phased Expansion

This proposal is deliberately scoped as a Kampala-only Year 1 pilot, so that the budget the funder is asked to approve is concrete, fully costed, and easy to verify, not a large multi-country ask built on assumptions. Expansion is a real ambition, but it is presented here as a phased outlook, to be re-budgeted once Year 1 results are in hand.

Phase	Geographic scope	Status
Year 1 (this proposal)	Kampala peri-urban areas - Kyebando-Kisalosalalo / Northern Bypass corridor and surrounding parishes	Fully costed: see Section 11 and the attached budget workbook
Year 2	2–3 further Uganda regions (e.g. Wakiso, Mukono, Jinja peri-urban areas), scaling toward ~24 camps/year	Indicative only - to be re-costed from Year 1 actuals
Year 3	First regional partnership outside Uganda (e.g. Rwanda or an eastern DRC border area), alongside continued Uganda operations	Indicative only - pending in-country partner assessment
Future vision	Full multi-country footprint: Uganda, Democratic Republic of Congo, Rwanda, Burundi, Tanzania, and potentially Malawi	Directional - not costed

Indicative Year 2 and Year 3 figures are included in the '3-Year Outlook (Indicative)' tab of the attached workbook, clearly marked as non-binding planning figures rather than a funding request.

7. Implementation Timeline

Month	Milestone
September 2026	Funding agreement signed; staff and CHW/VHT training workshop; equipment procurement; site confirmation for Camps 1–3; FCHIP outreach-module deployment
October 2026	Launch event with funding partner and community leaders; Camp 1 (Kampala peri-urban)
November 2026	Camp 2; first FCHIP data review; Quarter 1 report preparation begins
December 2026	Camp 3; Quarter 1 report to funder
January–March 2027	Camps 4–6; mid-point review of site selection and staffing; Quarter 2 report
April–June 2027	Camps 7–9; community feedback review; Quarter 3 report
July–September 2027	Camps 10–12; Year 1 evaluation; Year 2 concept and budget drafted with funder input

This timeline assumes a September 2026 start; if the agreement is finalised later, the same 12-camp structure shifts to an October 2026 start with the final camp falling in September/October 2027, as indicated by the funder as the latest acceptable start.

8. Organizational & Staffing Plan

Each camp day is staffed by a mix of paid clinical professionals, paid support roles, and stipend community volunteers, supported year-round by a small part-time coordination team:

Role	Camp-day team	Responsibility
Medical Doctor	1	Clinical oversight, complex cases, referral decisions
Clinical Officer	1	Consultations, treatment
Registered Nurses	3	Vitals, wound care, medication support
Physiotherapist / Sports Massage Therapist	2	Massage & physiotherapy station
Pharmacist / Dispenser	1	Medicine dispensing, stock control
Laboratory Technician	1	Point-of-care testing (glucose, etc.)
Triage Nurses / Assistants	2	Flow management, initial assessment
Health Educators / Counsellors	2	HIV testing & counselling, nutrition, family planning education
FCHIP Data Clerks	2	Digital registration and data capture
Program Coordinator / Logistics Lead	1	Overall camp-day management
Security	2	Crowd management, site security
CHW / VHT Volunteers	8	Mobilisation, queue support, follow-up liaison

Between camps, a part-time Program Manager, M&E/FCHIP Systems Officer, and Finance & Administration Support keep the program running: confirming sites, managing procurement, ensuring data quality, and preparing funder reports (see Section 11 for costing).

9. Technology & Data: The FCHIP Platform

Every Sports & Family Health encounter is recorded on FCHIP by trained data clerks using field tablets, whether or not connectivity is available at the venue, the platform's offline-capable mobile forms sync automatically once a connection is restored. This means the program is not just delivering one-off services; it is building a longitudinal, geographically mapped picture of family health need across Kampala's peri-urban communities, which can guide where future camps are held and which conditions need the most attention.

For the funder, FCHIP also solves a transparency problem common to outreach funding: instead of relying only on narrative reports, the funder can be given dashboard access showing real numbers, people screened, referrals made, conditions detected tied to the specific camps their funding supported.

10. Monitoring, Evaluation & Reporting

Program performance will be tracked against a small set of indicators, reported quarterly and available on request through FCHIP dashboards:

- Number of camps held against the 12-camp Year 1 plan
- Number of people registered and screened per camp and cumulatively
- Number of referrals made to FairBanks Medical Centre or other facilities, and the share of those referrals completed
- Number of positive screens for hypertension, diabetes, and other flagged conditions, and follow-up status
- Participant feedback / satisfaction from a short exit survey
- Cost per beneficiary reached, tracked against the USD 10.51 Year 1 planning estimate

11. Budget Overview

The full, itemised budget with every unit cost and quantity editable and formula-linked is provided in the accompanying Excel workbook. It totals approximately USD 50,440 for Year 1, covering 12 camps and reaching an estimated 4,800 people, for a cost of roughly USD 10.51 per person reached. The table below summarises the workbook's Annual Summary tab.

Budget line	Annual amount (USD)
Recurring camp-based program costs (12 camps)	34,170
Program management & overhead	5,400
Annual fixed costs (insurance, compliance, communications)	1,250
Subtotal — recurring annual program costs	40,820
One-time setup & equipment (Year 1 only)	6,320
Subtotal before contingency	47,140
Contingency (7%)	3,300
GRAND TOTAL: YEAR 1 FUNDING REQUEST	50,440

Within the recurring camp-based costs, spending breaks down by category as follows (annualised across 12 camps):

Category	Annual amount (USD)
Health workforce (camp-day facilitation)	7,860
Drugs & medical supplies	10,560
Sports massage & physiotherapy supplies	720
Hydration & refreshments	2,550

Category	Annual amount (USD)
Ambulance & emergency referral	2,160
Venue, logistics & equipment hire	6,660
Community mobilization & facilitation	2,760
FCHIP data & digital	480
Printing & registration materials	420

11.1 Budget justification, by category

Health workforce: day-rate honoraria for qualified doctors, clinical officers, nurses, physiotherapists, and pharmacists engaged for camp days, alongside modest stipends for CHW/VHT volunteers consistent with standard practice for short-term outreach facilitation in Uganda.

Drugs & medical supplies: a per-camp essential-drug kit and wound-care consumables sized for roughly 400 participants, plus point-of-care testing supplies (glucose strips, BP consumables) priced from typical Uganda wholesale pharmaceutical costs and to be confirmed against current supplier quotes, including FairBanks Outreach's existing partner, Denk Pharma.

Sports massage & physiotherapy: oils, liniments, tape, and recovery supplies for the station that most directly reflects the funder's own field a modest cost with high visibility and relevance.

Hydration & refreshments: drinking water, oral rehydration drinks, and juice for children and elderly participants, priced per bottle/serving for the full expected attendance.

Ambulance & emergency referral: a fully crewed ambulance (vehicle, driver, paramedic, fuel) on standby for the full camp day, priced consistently with private ambulance/medical-response providers operating in Kampala; if FairBanks' own ambulance can be deployed instead of a hired unit, this line reduces accordingly.

Venue, logistics & equipment hire: tents, tables, chairs, sound equipment, signage, and transport for staff and supplies the physical infrastructure of a one-day outreach event at a rotating venue.

Community mobilization & facilitation: staff and volunteer meals, a token facilitation for the hosting church, school, or sports centre, local permit fees, and photography/videography for funder reporting and visibility.

FCHIP data & digital: mobile data bundles and device upkeep for the ten field tablets used at each camp for digital registration.

Program management & overhead: a part-time Program Manager, M&E/FCHIP Systems Officer, and Finance & Administration Support, budgeted year-round rather than only on camp days, to provide continuity, procurement, data quality oversight, and funder reporting between camps.

One-time setup (Year 1 only): field medical equipment (BP machines, glucometers, thermometers, scales, stretchers), ten tablets for FCHIP data capture, branded staff wear, the FCHIP outreach-module deployment, an initial staff/CHW training workshop, and a launch event with the funding partner.

Contingency: 7% applied across the full Year-1 subtotal, to absorb reasonable price movement (fuel, drug pricing) and unplanned costs, standard practice for a first-year pilot budget.

11.2 A note on the original 'opiate' service line

A note on the original 'opiate' service line

The initial service list for this concept included 'opiate' alongside first aid, ambulance, massage, drinks, and triage. On review, FairBanks Medical Centre has deliberately not included controlled or narcotic opiate medicines in this budget or program design. Distributing narcotics at a community outreach camp requires a resident prescriber holding a controlled-drugs permit and specific licensing under Uganda's National Drug Authority and the Narcotic Drugs and Psychotropic Substances (Control) Act — this is not appropriate or legally permissible in a lay-volunteer/CHW-supported outreach setting. Instead, the Pharmacy & Pain Management station provides standard, non-narcotic pain relief (paracetamol, ibuprofen/NSAIDs) alongside the physiotherapy and massage station, which together address the great majority of pain-related needs at a sports-linked family camp. If the intention behind 'opiate' was a different specific service — for example IV fluids or oxygen therapy for emergencies — FairBanks would be glad to cost that in separately, through the correct regulatory pathway.

12. Risk Management & Compliance

- **Controlled-drugs compliance:** as above, no narcotic/opiate medicines are distributed at outreach camps; all dispensing follows National Drug Authority rules under a licensed pharmacist.
- **Outreach permits:** FairBanks will secure the necessary Ministry of Health and local-authority permits for each camp, budgeted under Annual Fixed Costs and Community Mobilization.
- **Child safeguarding:** school-based and family-oriented camps will follow a written child-safeguarding protocol, including staff briefing and a visible reporting point at every camp.
- **Data privacy:** FCHIP data collection will follow informed consent procedures appropriate for community health data, with access to identifiable records restricted to clinical and FCHIP staff.
- **Weather and logistics contingency:** camps at outdoor sports venues carry a rain/weather contingency (indoor backup space at the same venue where possible); the venue, logistics & equipment budget assumes tent/canopy hire accordingly.
- **Financial accountability:** quarterly financial and narrative reporting to the funder, with all budget assumptions and unit costs documented and open to verification against supplier quotes.

13. Sustainability & Long-Term Vision

Year 1 is designed to prove the Sports & Family Health component on a scale FairBanks can deliver and account for precisely. Beyond Year 1, sustainability rests on several tracks: continued and potentially expanded support from this funding partnership; diversification toward corporate and diaspora sponsors drawn in by a visible, sports-linked program; deeper integration with FairBanks' existing insurance partnerships (CIC, AAR, and upcoming Jubilee and Mutual), which could eventually support a modest cost-recovery model for non-outreach services; and the growing value of FCHIP's community health data itself, which can inform district health planning and attract public-health and research partners.

Geographically, the phased outlook in Section 6 reflects real ambition, first across Uganda, then into Congo, Rwanda, Burundi, Tanzania, and potentially Malawi but each phase will be earned through evidence from the one before it, not assumed in advance.

14. Conclusion & Call to Action

FairBanks Medical Centre already has the clinical team, the outreach track record, the community relationships, and the data platform this program needs. What is missing is the resource to connect all of it to the sports community our prospective partner has spent his career building. Sports & Family Health is designed to be that connection: a new FairBanks Outreach component built around sport, delivered by a trusted local provider on infrastructure that already works, and reported on transparently through data the funder can see for himself.

FairBanks proposes a short kick-off call to walk through this concept and the attached budget together, agree on Year 1 site selection, and confirm the September/October 2026 start date. We would also welcome the funder, or a representative of his initiative, at the Year 1 launch event.

15. Appendix: Contact & Attachments

FairBanks Medical Centre Limited

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Attachment: full Year 1 line-item budget, annual summary, category dashboard, budget notes and justification, and an indicative 3-year geographic outlook.